Adult advanced non-small-cell lung cancer (NSCLC) · ALK or ROS1 indication confirmed Adults with advanced ALK-positive or ROS1-positive NSCLC. National Cancer Control Programme (NCCP) 00243a is previously treated ALK disease; 00243b is ROS1 disease; 00243c is first-line ALK disease. Confirm the exact indication and local access; a listed indication, licence and reimbursement are different facts. This guide does not cover paediatric lymphoma or inflammatory myofibroblastic tumour.

NCCP 00243 · VERSION 7

Crizotinib monotherapy

The usual adult starting dose is 250 mg twice daily, every day, with no planned rest week. Use the strength and frequency on your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

BASELINE → TREATMENT → FOLLOW-UP

Crizotinib checks over time

Columns show sequence, not equal time periods. The team books tests and reviews results. NCCP “electrocardiogram (ECG) every 2 cycles” does not define an interval for this continuous oral treatment; agree the actual cadence. No patient-entered dates are needed.

Check	Before starting	First 2 months	After month 2	Ongoing benefit review	Hold / change / stop
Crizotinib treatment	Confirm adult indication, prescription and formulation	Daily treatment; no planned rest week	Continue authorised daily schedule	Review cancer benefit and tolerability	Confirm whether held, changed or stopped
Liver tests	Baseline alanine aminotransferase (ALT), aspartate aminotransferase (AST) and bilirubin	Weekly for first 2 months (SmPC); reconcile NCCP fortnightly wording	Monthly; more often if abnormal	Continue checks and symptom review	Review recovery before any authorised restart
Blood count and kidneys	Baseline full blood count (FBC) and renal profile	Monthly; earlier if fever, illness or dehydration	Monthly and when clinically needed	Review renal function and other medicines	Follow unresolved abnormalities
Heart and lungs	ECG/electrolytes near first dose; chest imaging and symptoms	Periodic ECG/electrolytes; pulse/ blood pressure (BP) and symptoms	Agree explicit ECG cadence; assess new symptoms promptly	NCCP chest X-ray monthly; disease imaging per consultant	Investigate breathlessness, fainting, swelling or rhythm symptoms
Vision, medicines and daily life	Review existing vision symptoms and every medicine/supplement	Report visual change; check nausea support and adherence	Symptoms, interactions, food and supply reviewed	Persistent/worsening vision symptoms need eye review	Severe visual loss needs urgent assessment; confirm medicines/ follow-up

THE WHOLE TREATMENT JOURNEY

Crizotinib monotherapy: treatment and checks

Adult ALK-positive or ROS1-positive advanced NSCLC

The usual adult starting dose is 250 mg twice daily, every day, with no planned rest week. Use the strength and frequency on your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First 2 months	After month 2	Ongoing benefit review	Hold / change / stop
Crizotinib treatment NCCP 00243 · SMPC	Confirm adult indication, prescription and formulation	Daily treatment; no planned rest week	Continue authorised daily schedule	Review cancer benefit and tolerability	Confirm whether held, changed or stopped
Liver tests NCCP 00243 · SMPC	Baseline ALT, AST and bilirubin	Weekly for first 2 months (SmPC); reconcile NCCP fortnightly wording	Monthly; more often if abnormal	Continue checks and symptom review	Review recovery before any authorised restart
Blood count and kidneys NCCP 00243 · SMPC	Baseline FBC and renal profile	Monthly; earlier if fever, illness or dehydration	Monthly and when clinically needed	Review renal function and other medicines	Follow unresolved abnormalities
Heart and lungs NCCP 00243 · SMPC	ECG/electrolytes near first dose; chest imaging and symptoms	Periodic ECG/electrolytes; pulse/BP and symptoms	Agree explicit ECG cadence; assess new symptoms promptly	NCCP chest X-ray monthly; disease imaging per consultant	Investigate breathlessness, fainting, swelling or rhythm symptoms
Vision, medicines and daily life NCCP 00243 · SMPC	Review existing vision symptoms and every medicine/supplement	Report visual change; check nausea support and adherence	Symptoms, interactions, food and supply reviewed	Persistent/worsening vision symptoms need eye review	Severe visual loss needs urgent assessment; confirm medicines/ follow-up

What each check is for

1 Eligibility and safe start

Before first dose

Sources: NCCP 00243 · SMPC

2 Early liver surveillance

Weekly for first 2 months

Sources: NCCP 00243 · SMPC

3 Monthly and symptom-led checks

Throughout treatment

Sources: NCCP 00243 · SMPC

4 Hold or stop safely

Toxicity, progression or changed priorities

Sources: NCCP 00243 · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.24 · Sources checked 2026-10-29

Source reconciled; independent clinical, pharmacy and patient/PPI review pending